

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

<think>

Finding Reasoning Paths :

1 . **Distract ibility and short attention span**:

Consider attention deficit-h yper activity disorder (AD HD) as a possible diagnosis , as these symptoms are commonly associated with ADHD .

2 . **Family history of major depressive disorder**:

Explore the possibility of com or bid conditions such as ADHD or bipolar disorder , which can involve emotional l ability .

3 . **Family history of Graves ' disease**:

Consider hyper thy roid ism as a potential underlying cause of hyper activity and distract ibility .

4 . **Re current otitis media**:

Explore potential genetic or metabolic conditions linked to otitis media , such as Pend red syndrome or dys ker atosis congen ita , which may be associated with hyper activity and distract ibility .

5 . **Behavior al symptoms (interr uption , hyper activity)**:

Evaluate for neuro development al disorders such as ADHD or other genetic synd rom es (e.g ., Smith -Magen is syndrome) that involve hyper activity and behavioral issues .

Reasoning Process :

1 . **Distract ibility and short attention span**:

The boy exhibits symptoms of distract ibility , short attention span , impuls ivity , and poor behavior , which are hallmark features of ADHD . ADHD is a common disorder in children and is often accompanied by com or bid conditions such as anxiety , emotional l ability , or behavioral issues . The family history of major depressive disorder in the mother raises the possibility of com or bid mood disorders , which can exacerbate ADHD symptoms .

2 . **Family history of Graves ' disease**:

Graves ' disease is associated with increased thyroid activity , which can cause hyper activity , distract ibility , and behavioral changes . However , the boy is 5 years old , and Graves ' disease is typically diagnosed in infants or young children . Additionally , Graves ' disease itself does not directly explain the cognitive symptoms like distract ibility and short attention span , which are more consistent with ADHD .

3 . **Re current otitis media**:

Rec urrent infections like otitis media are sometimes linked to genetic or metabolic conditions such as Pend red syndrome or dys ker atosis congen ita . These conditions are rare and typically involve multiple organ systems . Pend red syndrome , for example , is associated with hypo thy roid ism , hearing loss , and hyper activity , but the boy does not have hearing impairment or other signs of hypo thy roid ism . Dys ker atosis congen ita is linked to intellectual disability , which is not observed in this case . Therefore , these explanations are less likely .

4 . **Behavior al symptoms (interr uption , hyper activity)**:

The boy 's symptoms of hyper activity , impuls ivity , and poor behavior are consistent with ADHD . The presence of a family history of major depressive disorder in the mother suggests a potential for com or bid conditions , which can exacerb ate ADHD symptoms .

Conclusion :

The most appropriate next step in treatment is **behavior therapy** , as ADHD is the most likely diagnosis based on the boy 's symptoms of hyper activity , distract ibility , and short attention span . This approach addresses behavioral issues and helps manage ADHD symptoms . Additionally , given the family history of major depressive disorder , it may be beneficial to monitor for com or bid mood disorders such as anxiety or depression , which can impact treatment outcomes .

</think>

<answer>

Behavior therapy

</answer>

1.5

1.0

0.5

0.0

-0.5

-1.0

-1.5